

# Role-Play Scenario 1: Breaking Bad News - New Diabetes Diagnosis

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## Scenario Overview

**Setting:** Rural Primary Health Center waiting area **Time:** 10:00 AM, busy OPD day **Characters:**

- **Student Doctor:** Junior MBBS intern conducting screening camp
- **Patient (Role):** 42-year-old female teacher, Mrs. Lakshmi
- **Family Member (Role):** Patient's husband, supportive but anxious
- **Observer:** Faculty supervising the interaction

**Context:** Mrs. Lakshmi attended a community diabetes screening camp organized by medical students. Her random blood glucose was 245 mg/dL and confirmatory test showed HbA1c of 9.2%. She has no prior knowledge of diabetes.

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## Patient Profile (For Role-Player)

**Background Information:**

- Age: 42 years, married with 2 children (ages 16 and 13)
- Occupation: High school teacher, well-educated
- Monthly income: ₹35,000 (middle class)
- Health Status: Generally healthy, no significant past illnesses
- Family History: Father had "sugar problem" and passed away from heart attack
- Lifestyle: Sedentary job, home-cooked meals, occasional sweets during festivals

**Mental State During Scene:**

- Initially cooperative during screening
  - Shows growing anxiety as results are explained
  - Becomes tearful when hearing lifelong diagnosis
  - Expresses concerns about career, family, and lifestyle changes
  - Husband provides emotional support but asks many questions
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## Student Doctor Script Guide

**Phase 1: Setting the Scene (2 minutes)**

**Purpose:** Build rapport and confirm understanding

Doctor: "Hello Mrs. Lakshmi, I remember you from our diabetes screening camp. We've now received your blood test results. Is your husband comfortable staying with us during this discussion?"

## Phase 2: Giving the Diagnosis (3 minutes)

**Purpose:** Clearly communicate the diagnosis using SPIKES protocol

**S** (Setting - establish environment)

- Private, comfortable seating arrangement
- No interruptions, adequate time
- Tissues and water available

**P** (Perception - patient understanding)

Doctor: "Before we discuss the results, can you tell me what you understand about the test results we've shared with you?"

**I** (Invitation - permission to discuss)

Doctor: "Would it be okay if we discuss what these blood sugar tests mean for your health? I have some important information to share."

**K** (Knowledge - deliver diagnosis)

Doctor: "Mrs. Lakshmi, based on your symptoms and the tests we conducted, you have diabetes mellitus. This means your blood sugar levels are higher than normal. Your HbA1c is 9.2%, which indicates your average blood sugar over the past 3 months has been quite high."

**E** (Emotions - address feelings)

Doctor: "I can see this news is upsetting for you. It's completely normal to feel worried, scared, or even angry right now. Your feelings are valid."

**S** (Strategy & Summary - plan next steps)

Doctor: "We have several options to help you manage this well. The good news is that with proper treatment, most people with diabetes live healthy, active lives. Let me explain what we can do together."

## Phase 3: Education & Support (5 minutes)

**Purpose:** Provide basic information and coping strategies

Doctor: "Diabetes means your body isn't using sugar properly. We can help with:

- Medications to control blood sugar
- Dietary changes you're comfortable with
- Regular check-ups to monitor progress
- Support from family and doctors"

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## Key Communication Skills to Demonstrate

### Empathy & Active Listening

- Maintain eye contact and attentive posture
- Use verbal acknowledgments ("I understand this is hard news")
- Reflect patient's emotions ("You're feeling overwhelmed right now")
- Avoid interrupting the patient's concerns

### Clear, Non-Technical Language

- Avoid medical jargon or explain it immediately
- Use analogies the patient can relate to (teacher analogy)
- Ask for understanding check ("Does that make sense?")
- Encourage questions throughout

### Family Involvement

- Involve husband appropriately without overwhelming patient
- Address family member's questions directed to patient
- Encourage family support discussion

### Cultural Sensitivity

- Respect patient's status as educated teacher
- Acknowledge family history appropriately
- Consider work-related implications
- Respect privacy and dignity

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## Common Challenges & Solutions

### Challenge 1: Patient becomes very emotional

**Solution:** Allow expression of feelings, offer tissue, reassure that emotions are normal, continue when patient indicates readiness

### Challenge 2: Too much information at once

**Solution:** Prioritize diagnosis disclosure first, then ask what patient wants to know next, provide written information for later reading

### Challenge 3: Family member dominates conversation

**Solution:** Gently redirect questions to patient, explain importance of patient understanding first, offer separate discussion if needed

### Challenge 4: Patient denies diagnosis

**Solution:** Acknowledge normal defense mechanism, provide evidence from tests, emphasize benefits of early treatment

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## Assessment Rubric

### Communication Skills (40%)

- ☒ **Excellent (36-40 pts):** Empathy, active listening, clear language, appropriate pacing
- ☒ **Good (30-35 pts):** Most skills demonstrated, minor lapses in pacing/empathy
- ☒ **Developing (25-29 pts):** Skills present but inconsistent, some areas need improvement
- ☒ **Needs Practice (<25 pts):** Poor communication, lack of empathy, confusing information

### Content Accuracy (30%)

- Facts presented correctly with appropriate context
- Management options explained clearly
- Follow-up plan established

### Patient-Centered Care (20%)

- Patient concerns addressed adequately
- Family involved appropriately
- Cultural sensitivity demonstrated

### Professionalism (10%)

- Appropriate boundaries maintained
  - Respect for privacy and dignity
  - Time management effective
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## Debriefing Questions

### For the Student Doctor:

1. What was most challenging about this conversation?
2. How well do you think you addressed the patient's emotions?
3. What would you do differently next time?

### For the Observer/Faculty:

1. What communication strengths did the student demonstrate?
2. Were there opportunities for better patient-centered communication?

3. How might cultural factors affect this type of conversation?

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## Teaching Points for Faculty

### Breaking Bad News Skills:

- SPIKES protocol is essential for structured approach
- Emotional intelligence crucial for patient trust
- Non-verbal communication as important as verbal
- Documentation should include patient reactions and follow-up plan

### CBME Competencies Addressed:

- CS1: Patient-Provider Communication
  - PRO1: Ethical Practice (truth-telling)
  - PRO2: Cultural Competence
  - PC2: Patient Education & Counseling
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### Materials Needed:

- Patient information booklet (in English/Kannada)
- Glucometer and strips for demonstration
- Sample medication charts
- Appointment card for follow-up
- Referral slip if needed

**Duration:** 15-20 minutes **Group Size:** 1 student + 1 patient actor + 1 family member actor + 1 faculty observer **Objective:** Practice evidence-based approach to diagnosis disclosure and initial counseling